

Front

(L)297mm



ASPIRA Chewable Tablet 5 mg
ASPIRA Tablet 10 mg
ASPIRA Oral Granules 4 mg

TABLET DESCRIPTION

Aspira Chewable Tablet 5 mg
White to off-white, round, biconvex tablet with Pharmaniaga icon on one side and plain on the other.

Aspira Tablet 10 mg
Beige, square shaped biconvex coated tablet with Pharmaniaga icon on one side and plain on the other.

Aspira Oral Granules 4 mg
White free flowing granular powder.

COMPOSITION

Aspira Chewable Tablet 5 mg
Each chewable tablet contains Montelukast 5 mg

Aspira Tablet 10 mg
Each film coated tablet contains Montelukast 10 mg

Aspira Oral Granules 4 mg
Each sachet contains Montelukast 4 mg

ACTIONS

Pharmacodynamics
Montelukast is a selective and active leukotriene receptor antagonist. Montelukast inhibits bronchoconstriction due to antigen challenge. Montelukast is a selective leukotriene receptor antagonist of the cysteinyl leukotriene CysLT 1 receptor. The cysteinyl leukotrienes (LTC 4, LTD 4, LTE 4) are products of arachidonic acid metabolism that are released from various cells, including mast cells and eosinophils. They bind to cysteinyl leukotriene receptors (CysLT) found in the human airway. Binding of cysteinyl leukotrienes to leukotriene receptors has been correlated with the pathophysiology of asthma, including airway edema, smooth muscle contraction, and altered cellular activity associated with the inflammatory process, factors that contribute to the signs and symptoms of asthma.

It binds to cysteinyl leukotrienes (CysLT) type-1 receptors found in human airway (smooth muscle cells and macrophages), which prevents airway edema, smooth muscle contraction and other respiratory inflammation. The leukotrienes are also released from the nasal mucosa after allergen exposure where montelukast sodium may inhibit symptoms of allergic rhinitis.

Montelukast binding to the CysLT1 receptor is high-affinity and selective, preferring the CysLT1 receptor to other pharmacologically important airway receptors, such as the prostanoid, cholinergic, or beta-adrenergic receptor. Montelukast potently inhibits physiologic actions of LTC4, LTD4, and LTE4 at the CysLT1 receptor without any agonist activity.

Montelukast causes bronchodilation within 2 hours of oral administration; these effects were additive to the bronchodilation caused by a β -agonist.

Pharmacokinetics
Absorption
Montelukast is rapidly and nearly completely absorbed following oral administration. For the 10-mg film-coated tablet, the mean peak plasma concentration (C_{max}) is achieved 3 hours (T_{max}) after administration in adults in the fasted state. The mean oral bioavailability is 64%. The oral bioavailability and C_{max} are not influenced by a standard meal.

For the 5-mg chewable tablet, the C_{max} is achieved 2 hours after administration in adults in the fasted state. The mean oral bioavailability is 73%. Food does not have a clinically important influence with chronic administration.

Distribution
Montelukast is more than 99% bound to plasma proteins. The steady-state volume of distribution of montelukast averages 8 to 11 liters.

Metabolism
Montelukast is extensively metabolized in the liver by cytochrome P450 isoenzymes, mainly by CYP2C8 and to a lesser extent by CYP3A4 and CYP2C9. Therapeutic plasma concentrations of montelukast do not inhibit cytochromes P450 3A4, 2C9, 1A2, 2A6, 2C19, or 2D6.

Elimination
The plasma clearance of montelukast averages 45 mL/min in healthy adults. Following an oral dose of radiolabeled montelukast, 86% of the radioactivity was recovered in 5-day fecal collections and <0.2% was recovered in urine. Coupled with estimates of montelukast oral bioavailability, this indicates montelukast and its metabolites are excreted almost exclusively via the bile.

The mean plasma half-life of montelukast ranged from 2.7 to 5.5 hours. The pharmacokinetics of montelukast are nearly linear for oral doses up to 50mg. There was no difference in pharmacokinetics between dosing in the morning or in the evening. Only a little accumulation of parent drug in plasma (~14%) on once daily dosing of 10mg montelukast.

INDICATION

For the prophylaxis and chronic treatment of asthma in adults and pediatric patients 12 months of age and older.

Montelukast is indicated in adults and pediatric patients 2 years of age and older for the relief of daytime and nighttime symptoms of seasonal allergic rhinitis.

CONTRAINDICATIONS

Hypersensitivity to montelukast or any component of the formulation.

RECOMMENDED DOSAGE

Montelukast should be taken once daily. For asthma, the dose should be taken in the evening. For allergic rhinitis, the time of administration may be individualized to suit patient needs.

Patients with both asthma and allergic rhinitis should take only one tablet daily in the evening.

• *Adults 15 Years of Age and Older with Asthma and/or Seasonal Allergic Rhinitis*
The dosage for adults 15 years of age and older is one 10-mg tablet daily.

- *Pediatric Patients 6 to 14 Years of Age with Asthma and/or Seasonal Allergic Rhinitis*
The dosage for pediatric patients 6 to 14 years of age is one 5-mg chewable tablet daily.
- *Pediatric Patients 2 to 5 Years of Age with Asthma and/or Seasonal Allergic Rhinitis*
The dosage for pediatric patients 2 to 5 years of age is one packet of 4-mg oral granules daily.
- *Pediatric Patients 12 months to 2 Years of Age with Asthma*
The dosage for pediatric patients 12 months to 2 years of age is one packet of 4-mg oral granules daily.

Administration of oral granules
Montelukast oral granules can be administered either directly in the mouth, or mixed with a spoonful of cold or room temperature soft food (e.g., applesauce) or dissolved in 1 teaspoonful (5 ml) of cold or room temperature baby formula or breast milk. The packet should not be opened until ready to use. After opening the packet, the full dose of Montelukast oral granules must be administered immediately (within 15 minutes). If mixed with food or dissolved in baby formula or breast milk, Montelukast oral granules must not be stored for future use. Montelukast oral granules are not intended to be dissolved in any liquid other than baby formula or breast milk for administration. However, liquids may be taken subsequent to administration.

General Recommendations
The therapeutic effect of Montelukast on parameters of asthma control occurs within one day. Montelukast tablets, chewable tablets, and oral granules can be taken with or without food. Patients should be advised to continue taking Montelukast while their asthma is controlled, as well as during periods of worsening asthma. No dosage adjustment is necessary for pediatric patients, for the elderly, for patients with renal insufficiency, or mild-to-moderate hepatic impairment, or for patients of either gender.

Montelukast is a long term-controller medication which should not be substituted for short acting beta-agonists. It is effective alone or in combination with other prophylactic agent. Montelukast is a preventive agent, which should be used in addition to other drugs for the management of asthma.

Therapy with Montelukast in Relation to Other Treatments for Asthma
Montelukast can be added to a patient's existing treatment regimen.

Reduction in Concomitant Therapy :

Bronchodilator Treatments
Montelukast can be added to the treatment regimen of patients who are not adequately controlled on bronchodilator alone. When a clinical response is evident (usually after the first dose), the patient's bronchodilator therapy can be reduced as tolerated.

Inhaled Corticosteroids
Treatment with Montelukast provides additional clinical benefit to patients treated with inhaled corticosteroids. A reduction in the corticosteroid dose can be made as tolerated. The dose should be reduced gradually with medical supervision. In some patients, the dose of inhaled corticosteroids can be tapered off completely. Montelukast should not be abruptly substituted for inhaled corticosteroids.

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ARTWORK LOG		
Version no.	Date	Reason for Change
01	12.04.18	- Amend wording and PRP no.

02	23.04.18	- Amend wording.
03	30.04.18	- Amend wording.
04	14.06.19	- Amend wording.
05	17.06.19	- Add ® beside Pharmaniaga logo.
06	19.06.19	- Remove the word "sodium"
07	25.06.19	- Amend wording.
08	02.07.19	- Amend wording.
09	19.12.19	- Amend wording.
10	01.09.2020	- Amend wording and revision date.
11	07.12.2020	- Amend wording.
12	10.12.2020	- Amend wording.
13	30.09.2021	- Amend wording and PRP from V11 artwork.
14	23.09.2022	- Amend wording and PRP.
15	07.12.2022	- Amend wording and PRP.

MODE OF ADMINISTRATION

Oral.

WARNINGS AND PRECAUTIONS

The efficacy of oral Montelukast for the treatment of acute asthma attacks has not been established. Therefore, oral Montelukast should not be used to treat acute asthma attacks. Patients should be advised to have appropriate rescue medication available.

While the dose of concomitant inhaled corticosteroid may be reduced gradually under medical supervision, Montelukast should not be abruptly substituted for inhaled or oral corticosteroids. Neuropsychiatric events (eg. agitation, aggression, anxiousness, dream abnormalities, hallucinations, depression, disorientation, insomnia, irritability, restlessness, suicidal thinking and behavior (including suicide), tremor) have occurred. Since other factors may have contributed to these events, it is not known if they are related to Montelukast. Physicians should discuss these adverse experiences with their patients and/or caregivers. Patients and/or caregivers should be instructed to notify their physicians if these changes occur.

The reduction in systemic corticosteroids dose in patients receiving anti-asthma agents including leukotriene receptor antagonists has been followed in rare cases by the occurrence of one or more of the following: eosinophilia, vasculitic rash, worsening pulmonary symptoms, cardiac complications, and/or neuropathy sometimes diagnosed as Churg-Strauss syndrome, a systemic eosinophilic vasculitis.

Although a casual relationship with leukotriene receptor antagonism has not been established, caution and appropriate clinical monitoring are recommended when systemic corticosteroid reduction is considered in patients receiving Montelukast.

Patients with known aspirin sensitivity should continue avoidance of aspirin or non-steroidal anti-inflammatory agents while taking Montelukast.

Although Montelukast is effective in improving airway function in asthmatics with documented aspirin sensitivity, it has not been shown to truncate bronchoconstrictor response to aspirin and other documented aspirin sensitivity.

Effects on Ability to Drive and Use Machines

Montelukast is not expected to affect a patient's ability to drive a car or operate machinery. However, in very rare cases, individuals have reported drowsiness or dizziness.

For Aspira Chewable Tablet 5mg
WARNING: Unsuitable for Phenylketonurics.

PREGNANCY AND LACTATION

Pregnancy

The safety and efficacy of montelukast have not been determined in pregnant women. Rarely, congenital limb defects have been reported during post marketing surveillance; however, causality has not been established. Due to the lack of human safety information, montelukast should be used in pregnant women only if the potential benefit outweighs the potential risk to the fetus.

Lactation

Lactation studies with montelukast in humans have not been conducted. Montelukast is excreted into the milk of lactating rats. Until further data are available, caution is advised if montelukast is used in nursing women. Available evidence and/or expert consensus is inconclusive or is inadequate for determining

infant risk when used during breastfeeding. Weigh the potential benefits of drug treatment against potential risks before prescribing this drug during breastfeeding.

DRUG INTERACTION

Montelukast may be administered with other therapies routinely used in the prophylaxis and chronic treatment of asthma, and in the treatment of allergic rhinitis.

Montelukast is not anticipated to alter the metabolism of drugs metabolized by this enzyme (e.g., paclitaxel, rosiglitazone, repaglinide).

Concurrent use of gemfibrozil and montelukast may result in elevated montelukast plasma concentrations.

Concurrent use of prednisone and montelukast may result in severe peripheral edema.

Concurrent use of montelukast and repaglinide may result in increased repaglinide plasma concentrations.

Montelukast is metabolised by CYP 3A4, caution should be exercised, particularly in children, when montelukast is co-administered with inducers of CYP 3A4, such as phenytoin, phenobarbital and rifampicin.

Concurrent use of montelukast and phenobarbital may result in decreased bioavailability of montelukast. Concurrent use of montelukast and rifampin may result in decreased bioavailability of montelukast.

ADVERSE REACTION

Montelukast has been generally well tolerated. Side effects, which usually were mild, generally did not require discontinuation of therapy.

Cardiovascular Effects: palpitations, allergic granulomatous angiitis (Systemic eosinophilia with vasculitis and a clinical presentation consistent with Churg-Strauss. Symptoms may include eosinophilia, vasculitic rash, worsening pulmonary symptoms, cardiac complications and/or neuropathy).

Skin and subcutaneous tissue disorders: angioedema, bruising, erythema nodosum, atopic dermatitis, eczema, infection of skin and /or subcutaneous tissue, rash, urticaria, erythema multiforme and pruritus.

Gastrointestinal Effects: abdominal pain, dental pain, diarrhea, gastroenteritis, indigestion, infection of tooth, nausea, pancreatitis, tonsillitis, vomiting and dyspepsia.

Hematologic Effects: blood coagulation disorder (increased bleeding tendency).

Hepatic Effects: ALT/ AST levels raised, cholestatic hepatitis, hepatocellular and mixed-pattern liver injury.

Immunologic Effects: hypersensitivity reactions, including anaphylaxis, varicella and viral infection. Very rarely hepatic and eosinophilic infiltration.

Neurologic Effects: asthenia, disorientated, dizziness, headache, hyperactive behavior, hypesthesia, insomnia, paraesthesia, seizure, sinus headache, drowsiness, and tremor.

Ophthalmic Effects: conjunctivitis and myopia.

Otic Effects: otalgia, otitis and otitis media.

Psychiatric Effects: aggressive behavior, agitation, altered behavior, anxiety, depression, dream disorder, feeling nervous, hallucinations, irritability, nightmares, restlessness, sleep disorder, somnambulism, suicidal thinking and behavior (including suicide), hostility, anxiousness, disturbances in attention, memory impairment and tic.

Renal Effects: pyuria and enuresis in children.

Respiratory Effects: bronchitis, acute, cough, epistaxis, laryngitis, nasal congestion, nasal discharge, pharyngitis, pneumonia, respiratory tract infection, rhinitis, sinusitis, upper respiratory infection, wheezing and pulmonary eosinophilia.

Musculoskeletal and connective tissue disorders: arthralgia and myalgia including muscle cramps.

Other: fatigue, fever, influenza, malaise, traumatic injury, edema and pyrexia.

Postmarketing Experience

Blood and lymphatic system disorders: Thrombocytopenia.
Psychiatric disorders: Obsessive-compulsive symptoms.

OVERDOSAGE

Symptoms

Symptoms included abdominal pain, somnolence, thirst, headache, vomiting and psychomotor hyperactivity.

Treatment

Treatment is symptomatic and supportive. Treatment may include removal of unabsorbed material from the gastrointestinal tract, clinical monitoring and supportive therapy if required. It is not known if montelukast can be removed by peritoneal dialysis or hemodialysis.

STORAGE CONDITION

Store below 30°C. Protect from light.

SHELF-LIFE

Products should not be used beyond the expiry date imprinted on the product packaging.

PRESENTATION

Aspira Chewable Tablet 5 mg
In blister of 28 tablets.

Aspira Tablet 10 mg
In blister of 28 tablets.

Aspira Oral Granules 4 mg
A carton of 28 sachets.

Date of revision: 07th December 2022

PRODUCT REGISTRATION HOLDER / MANUFACTURER :
PHARMANIAGA MANUFACTURING BERHAD (198001006232)
No. 11A, Jalan P/1, Kawasan Perusahaan Bangi,
43650 Bandar Baru Bangi, Selangor Darul Ehsan, Malaysia

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